EL NIÑO ACTION BRIEFING · 8 OCTOBER 2026

Juneau Town, downtown Milwaukee

El Niño usually gives Milwaukee a warmer, less snowy winter, and that is the trap. People here die of cold on ordinary nights and early in the season, not at −40°F, when the whole system is already mobilised. They die alongside a much larger overdose toll. The most important step is to make every warming room a naloxone site and open it from the first freezing night.

IMPACT REGION	EXCESS HEAT DEATHS	WINDOW	DOMINANT HAZARD
Milwaukee County, WI (inferred)	None published for the US, Wisconsin, or the county	n/a here; global 6-month figure 239,000 ± 11,000	Cold exposure of unsheltered and isolated people on moderate nights, compounded by overdose

12/12 recommendations checked. 7 corrected, 0 confirmed as written, 4 unverified, 1 contraindicated (parametric cover and international anticipatory funds, which do not operate here). None survived exactly as proposed. The four funding placements were checked separately: 2 contraindicated (buying naloxone kits the state already gives free, and a furnace top-up WHEAP already pays for) and 2 unverified. **The funding target fell 51%**, from $405k to about $200k, past the one-third flag. **Read the unverified count with care:** the shared web-search budget ran out during verification, and the City

EXECUTIVE SUMMARY — AT A GLANCE



43.04°N 87.90°W, United States of America · Natural Earth

- **The risk.** People die of cold here on ordinary nights early in the season, alongside a much larger overdose toll; El Niño's warmer, less snowy winter is the trap.
- **First action.** The Milwaukee Health Department and Vivent Health LifePoint make every warming room, shelter, and drop-in a staffed naloxone site using free state kits, stocked by 1 November.*
- **The money.** $50k floor (rising to $150k once the warming placement is confirmed), about $200k target, about $400k ceiling; the floor pays for naloxone distribution staff.
- **Do not.** Do not clear encampments between October and March.

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* Suggested formulation: check with a health worker before sharing.

and County websites returned 403 errors, so several Milwaukee facts were checked against news reports rather than agency records. Exposure: Climate Impact Lab, 451,000 at Risk, September 2026, pre-peer-review. It publishes no figure for the United States, Wisconsin, or Milwaukee County. Heat is not this place's winter hazard, so no population-scaled figure is given.

Read this first: the warm forecast is the danger, and cold is not the biggest killer. Two findings frame everything below. First, five of seven lenses found that cold deaths here happen on ordinary nights. A 72-year-old died one block west of the river on Thanksgiving night 2021 at 19°F. Five people aged 56 to 82 died between 27 November and 13 December 2024. Three unhoused men died in a January 2024 cold snap during the last strong El Niño. A mild-winter forecast lowers everyone's guard. Second, hypothermia kills about 10 people a year countywide, while fatal overdoses ran at 387 in 2025, down from 674 in 2022. Cold and overdose meet in the same people and the same rooms: a 7-day average of 0°C carries 1.3 times the odds of fatal opioid overdose compared with 11°C (Goedel et al., 2019). So the plan pairs them.

WHAT THIS PLACE SHOULD DO — RANKED

Actions are ranked by expected lives saved per unit of effort before the peak. The backing lenses are shown on each action, and the RACI chart below assigns each one. An action several lenses reached independently is the stronger bet, but only where its evidence came from somewhere like here. One rule underlies every action: plan for the 19°F night with nobody looking, not the −40°F night when everyone is.

1. Make every warming room, shelter, and drop-in a staffed naloxone site, using free state kits

PARTIALLY CONFIRMED | PUBLIC HEALTH · PHILANTHROPIST · FRONTLINE · ECONOMIST | INVENTORY: LOOSE ONLY (SUBSTANCE-USE REGULATION)

OWNER	BY WHEN	COST	PREVENTS
Milwaukee Health Department and Vivent Health LifePoint, supplying the warming-site network	Naloxone Direct applications by 25 Oct; sites stocked by 1 Nov	Kits free; ~$60–90k for distribution staff, training, and wall boxes (verifier estimate)	Overdose deaths among people using alone indoors in cold weather

* Suggested formulation: check with a health worker before sharing.

Wisconsin DHS's Naloxone Direct programme gives naloxone free to local health departments and syringe services programmes, and applications for 2026–27 are due **25 October 2026**. Shelters cannot apply directly, so they need MHD or LifePoint to supply them. That makes distribution, not kits, the binding constraint. The action: every warming room and shelter holds naloxone, staff are trained, boxes are mounted, and restocking is scheduled, with the message “don't use alone, don't sleep alone outside.” Extend it to transit hubs and to downtown bars and hotels.*

Transferability: the mortality evidence is pre-fentanyl. In Massachusetts, 2002–09, community naloxone programmes were associated with overdose death rate ratios of 0.73 at low uptake and 0.54 at high uptake (Walley et al., *BMJ* 2013, observational). Coffin & Sullivan (*Ann Intern Med* 2013) modelled 1 death averted per 227 kits among heroin users. Fentanyl overdoses are faster and more often unwitnessed, which weakens the transfer but strengthens the case for placing naloxone where people are together. The cold–overdose link is confirmed in two studies; the “using alone indoors” mechanism is a plausible reading, not a finding. The 2025 county overdose figures come from the county's own release via the lenses; the verifier could not reach the county site.

2. Start the nightly warming season at the first freezing night, not late November, and put it in the budget

PARTIALLY CONFIRMED | EMERGENCY MGR · SKEPTIC · FRONTLINE · ECONOMIST · PHILANTHROPIST · PUBLIC HEALTH | INVENTORY: NONE FOUND

OWNER	BY WHEN	COST	PREVENTS
Milwaukee Coalition on Housing and Homelessness (MCHH) warming network, with County Housing Services and the City	**Season open by the first sub-freezing night (Oct–Nov); budget line before the 2027 cycle**	**Not published; held at $100k, conditional (see funding)**	**Early-season deaths like the five in late Nov–mid Dec 2024**

Most of this already exists, and the lenses missed it. In 2024–25 and 2025–26 the overnight sites ran *every night* from 7pm to 7am through the end of March, not on a temperature trigger. The temperature thresholds the lenses cited (20°F; 25°F at St. Ben's in January 2024) belong to earlier seasons. The gap that remains is the start: the season opened about 25 November 2024 and St. Ben's opened 1 December 2025. Five probable hypothermia deaths fell between 27 November and 13 December 2024. The action is to open from the first sub-freezing night and to replace annual fundraising with a recurring City and County line. In January 2024, a city source said warming shelters were “not included in the city budget”.

* Suggested formulation: check with a health worker before sharing.

Transferability: moderate cold is the larger killer in the multi-country data (Gasparrini et al., *Lancet* 2015) and in Toronto, where hypothermia odds rose 1.64-fold per 5°C drop in daily minimum and most events fell in moderate weather (Zhang et al., 2019). No study shows warming centres reduce deaths; this is plausible, not proven. The late-2024 death cluster, the budget quote, and the $25k Greater Milwaukee Foundation figure are from news reports the verifier could not re-check against Medical Examiner records. No 2026–27 plan had been announced by 8 October; the 2024 plan was announced on 18 November.

3. Open one low-barrier overnight site east of the river that takes couples, pets, and belongings without a call ahead

PARTIALLY CONFIRMED | EMERGENCY MGR · SKEPTIC · FRONTLINE | INVENTORY: NONE FOUND

OWNER	BY WHEN	COST	PREVENTS
County Housing Services and MCHH; Cathedral Center (845 N Van Buren) as the most plausible host or partner	15 Nov	Staffing within the warming line; no figure published	People who will not split up, or who have no phone, sleeping out

In 2025–26 there was no overnight site downtown east of the river. Five of six sites were on the near west and north sides, and the sixth, Unity Lutheran, is in Bay View about four miles south. Sites were split by type: the Rescue Mission took men and Joy House women and families, and both required a call; Guest House took men only; Repairers of the Breach and Unity took single adults. No site was found that took couples or pets. Cathedral Center, inside Juneau Town, already shelters women and families and is the obvious partner to ask.

* Suggested formulation: check with a health worker before sharing.

Transferability: the case rests on the published site rules, not on a documented turn-away. Milwaukee has no recorded case of a couple or pet owner refused. Cathedral Center's current capacity was not checked. Repairers of the Breach's own site now describes only a daytime shelter, so its overnight role should be confirmed.

4. Pre-map where people sleep after dark and run voluntary outreach rounds on every cold-warning night

PARTIALLY CONFIRMED | EMERGENCY MGR · FRONTLINE · HISTORIAN · ECONOMIST | INVENTORY: NONE FOUND

OWNER	BY WHEN	COST	PREVENTS
County Housing Services street outreach, with Milwaukee Downtown BID #21 ambassadors and Street Angels	**Map by 1 Dec; rounds every cold-warning night**	**Existing teams; $50k matched surge for transport and overtime (lens estimate)**	**Deaths under bridges, in cars, and on heating vents**

The January 2024 dead were found under a bridge, in a car, and by a heating vent near railroad tracks (WTMJ, TMJ4). The action lists those kinds of places downtown, including vents, skywalk doors, and the lakefront, and assigns each to an evening round on warning nights. Transport to shelter is voluntary, and police come only on request. Two distribution points already reach people. The Open Door Café at the Cathedral of St. John (831 N Van Buren) serves lunch Sunday to Friday, 11:15am–12:45pm, and hands out hygiene kits. Street Angels runs outreach buses two nights a week with 70–90 stops a night, under bridges and at encampments. Use both for dry socks, layers, and cards with the warming-site list and 211.

* Suggested formulation: check with a health worker before sharing.

Transferability: no controlled evidence shows cold-night outreach reduces deaths; the case is face validity. A round only helps if beds are free that night. Unverified: the January 2024 death details (lens sources, not re-checked), the 61 unsheltered in the January 2024 point-in-time count, the county's "92% fewer unsheltered" claim (a county-reported trend, not an evaluation), and whether BID #21 still has a homeless-outreach ambassador.

5. Reconnect disconnected households before 1 November, and have the PSC route still-dark addresses to people who knock on doors

PARTIALLY CONFIRMED | ECONOMIST · SKEPTIC · HISTORIAN · EMERGENCY MGR | INVENTORY: NONE FOUND

OWNER	BY WHEN	COST	PREVENTS
WHEAP via 1-866-432-8947 (local agency to be confirmed); We Energies; Wisconsin PSC; Milwaukee Health Department	**31 Oct for reconnections; PSC request now, ahead of the 15 Nov report**	**WHEAP benefits already appropriated; PSC order near zero**	**Older people dying at home without heat**

The winter rule is stronger than the lenses said and still leaves a hole. PSC 113.0304 covers all occupied residences from 1 November to 15 April. A utility may disconnect only a household above 250% of the poverty line in which nobody's health is endangered by age, disability, or infancy. It does not force reconnection, except that a utility must "immediately restore service" if it sees a danger to health or life during its fall contacts. Under PSC 113.0303, large utilities must try to contact every premises disconnected in the past 12 months by 15 October, with a personal visit by 25 October. They report *counts* to the PSC by 15 November and weekly after that. The rule already requires utilities to "cooperate with... social service agencies." The lever is a PSC order that turns that duty into a weekly list of still-dark addresses sent to the Health Department, which already checks on people. In January 2019, during a weak El Niño, a 38-year-old woman died in her unheated apartment after a thermostat failed.

* Suggested formulation: check with a health worker before sharing.

Transferability: the rule text is primary-sourced (PSC 113, November 2025 register). Expect a fight over customer privacy. Unverified: the $1,200 WHEAP crisis cap for Milwaukee (it was confirmed statewide in the Lake Geneva run the same day), the ~3,804 homes still disconnected in 2021, the Zondra Strong case, and which agency runs WHEAP in Milwaukee County (UMOS, Community Advocates, or SDC). No-heat checks of older tenants by the Department of Neighborhood Services were proposed but could not be verified (the city site returned 403).

6. Add cold triggers to the plans Milwaukee already has, rather than building a new task force

UNVERIFIED | HISTORIAN · EMERGENCY MGR | INVENTORY: CHANGE IN FORECAST QUALITY (SHRADER ET AL. 2023), INDIRECT

OWNER	BY WHEN	COST	PREVENTS
Milwaukee County Office of Emergency Management and the Health Department; Medical Examiner for a daily probable-cold-death feed	1 Dec	Staff time	A repeat of the same death sites going unnoticed

After the July 1995 heat wave killed 91 people in Milwaukee County, a multi-agency heat plan followed, and heat deaths were 11 in 1999 (Weisskopf et al., *AJPH* 2002). The Historian's point is that no equivalent exists for cold. The verifier's correction: first check whether the existing heat and emergency plans already carry cold triggers, and add them there. Separately, get Emergency Management and Housing Services a daily feed of probable cold deaths, with location, from the Medical Examiner's existing case data, so the next night's rounds can be redirected.

* Suggested formulation: check with a health worker before sharing.

Transferability: the heat precedent is local, but its attribution is weak. The authors say preparedness “may also have contributed”. Whether a Milwaukee cold plan or Code Blue protocol already exists could not be checked, because the city site returned 403. That redundancy test is the whole question. Hypothermia deaths take days to confirm, so a 24-hour flag can only be “probable”.

7. Give ice cleats to older adults who have already fallen

PARTIALLY CONFIRMED (VERIFIED IN THE LAKE GENEVA RUN) | PUBLIC HEALTH | INVENTORY: NONE FOUND

OWNER	BY WHEN	COST	PREVENTS
Milwaukee County Department on Aging	**1 Dec**	***Unfunded*, small**	**Outdoor falls on freeze–thaw ice**

A Wisconsin trial in community-dwelling people 65+ who had fallen in the past year cut outdoor falls with Yaktrax-type cleats (RR 0.45; 0.42 on snow and ice days; McKiernan, *J Am Geriatr Soc* 2005, n=113). Wisconsin has the highest US fall-death rate for people 65 and over (158.4 per 100,000 in 2023). The January 2025 death of an 80-year-old who fell on a sidewalk at 12th and Orchard and never got up shows the overlap between falls and cold.

Transferability: same state, small unblinded trial, falls not deaths. No study links warm El Niño winters to more falls. Only about 11% of downtown residents are 65 or older, by a weak commercial estimate with an undefined boundary.

* Suggested formulation: check with a health worker before sharing.

WHO DOES WHAT

RACI CHART

R = Responsible (does the work) · **A** = Accountable (one per action; signs off and answers for it) · **C** = Consulted (asked before) · **I** = Informed (told after). Built from the owners named in the verified actions. No one here has been contacted or has agreed to a role; every row is a proposal.

Action	Milwaukee Health Dept	Vivent Health LifePoint	MCHH warming-site network	County Housing Services	Cathedral of St. John & Cathedral Center	Street Angels	Downtown BID #21	County Emergency Mgmt	Medical Examiner	Wisconsin PSC	We Energies	WHEAP agency	County Dept on Aging	Greater Milwaukee Foundation
1. Naloxone at every warming site	**A**	**R**	**R**	**C**	**C**	**R**	**I**		**I**					**C**
2. Warming season from first freeze; budget line	**R**		**R**	**A**	**C**		**I**	**C**						**C**
3. Low-barrier site east of the river	**C**	**C**	**R**	**A**	**R**		**C**							**C**
4. Mapped outreach rounds on cold nights	**C**	**C**	**C**	**A/R**	**R**	**R**	**R**	**I**	**C**					
5. Reconnect before 1 Nov; PSC list to MHD	**R**			**I**						**A**	**R**	**R**	**C**	
6. Cold triggers in existing plans; ME feed	**R**		**C**	**C**				**A**	**R**					
7. Ice cleats for older fallers	**C**												**A/R**	

What the chart shows. **The Health Department sits on almost every row**, because naloxone supply, warming policy, and home-heat checks all route through it. Its capacity is the plan's biggest dependency. **County Housing Services is accountable for the three shelter and outreach rows**, and the MCHH network, which actually runs the sites, has no single accountable owner of its own. **The Cathedral is the only Juneau Town institution with an R**: every other provider in the network sits west of the river, which is the geographic gap action 3 exists to close. **The PSC is accountable for row 5 but does no work**; the order costs it nothing, and the utilities and the Health Department carry the load.

WHERE MONEY DOES THE MOST

FUNDING

The Philanthropist lens, reconciled against the ranked actions and verified. Placements are ranked by marginal lives saved per dollar. The envelope is bounded by what this place can spend before the peak, not by what it needs.

FLOOR
$50k (rising to $150k once row 2 is confirmed)

TARGET
~$200k

CEILING
~$400k

LARGEST UNCERTAINTY
Whether 2026–27 warming nights and their start date are already funded

Floor: $50k for naloxone distribution staff, which is verified, cheap, and pairs with free kits. It rises to $150k once the warming placement is confirmed not to duplicate existing funding. **Target:** $50k for naloxone staff, $100k for warming, and $50k for outreach, which totals $200k.

* Suggested formulation: check with a health worker before sharing.

Ceiling: $100k for naloxone staff, $200k for warming (the lens's figure for using Guest House's offered 25–30 extra beds, unverified), and $100k for outreach, which totals $400k. The target fell 51% from the lens's $405k, because $205k of kit purchase and a $50k WHEAP top-up both duplicate free or funded programmes.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
Milwaukee Health Dept and Vivent Health LifePoint	Action 1: distribution staff, training, wall boxes, restocking at warming sites	$50k (was $205k for kits)	Direct grant	Kits free via DHS Naloxone Direct; Coffin & Sullivan 2013 base case 1 death averted per 227 kits (modelled, pre-fentanyl). Staff cost is a verifier estimate ($60–90k range)	PARTIALLY CONFIRMED
MCHH warming-site operators (St. Ben's and partners)	Actions 2–3: earlier season start and an east-of-river low-barrier site	$100k, held	Direct grant, conditional	Lens precedent: a $100k We Energies gift once funded up to 125 walk-in adults, or about $800 per adult place for the season (gift not re-verified). Release only if 2026–27 nights are not already funded	UNVERIFIED
County Housing Services street outreach	Action 4: transport and overtime on cold-warning nights	$50k	Matching grant, invoice-backed	Lens estimate; no published cost	UNVERIFIED
~~Vivent LifePoint and MHD: 5,000 kits × $41~~	~~Pre-positioned naloxone stock~~	~~$205k~~	n/a	The state already supplies these exact recipients free; applications due 25 Oct	CONTRAINDICATED
~~WHEAP agency: furnace and bulk-fuel top-up~~	~~Crisis top-up~~	~~$50k~~	n/a	WHEAP's HVAC programme already repairs or replaces failed heating; fund only a documented gap such as renters	CONTRAINDICATED

The funder's one thing: the weather is not what kills most people here; overdose is. A grant shaped around El Niño funds blankets for a hazard that kills about 10 people a year and misses one that killed 387 last year. The cheapest life saved is a trained person with a free naloxone kit in a warm room. Money spent on the kits themselves is wasted, because the state gives them away.

* Suggested formulation: check with a health worker before sharing.

DO NOT DO THIS CONTRAINDICATED

Advice that circulates for this hazard and would cost lives or waste the window here.

Buy naloxone kits

CONTRAINDICATED AS A PURCHASE

The Philanthropist's largest placement was $205k for 5,000 kits. Wisconsin DHS's Naloxone Direct programme already gives naloxone free to the Health Department and to syringe services programmes like LifePoint, and DHS runs free vending machines and mail order as well.

Instead: file the Naloxone Direct applications by 25 October and pay for the people, boxes, and restocking that get kits into warming rooms (action 1).

Clear encampments between October and March

AVOID (SKEPTIC; NOT SEPARATELY VERIFIED)

In 2019 the state set a 31 October eviction deadline for the Tent City under I-794, on Juneau Town's southern edge, and the deadline slipped while tents sat under snow. Moving people in winter breaks the outreach relationships that action 4 depends on, and it pushes people toward bridges, cars, and vents, where the January 2024 deaths occurred.

Instead: run outreach to camps on every freezing night, with a real bed offer and transport (actions 3 and 4).

Tell people it will be a mild El Niño winter

AVOID (SKEPTIC, EMERGENCY MGR, HISTORIAN)

El Niño does tilt Milwaukee warm: about 10 inches less snow than the ~50-inch normal, and 2015–16 ran 4–5°F above normal. But the −20°F night of 30–31 January 2019 fell in a weak El Niño winter, and three people died in January 2024 during a strong one. “Mild” is how guards come down.

Instead: say “likely milder on average, with dangerous cold nights; most cold deaths here happen on ordinary nights.”

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
July 1995	Heat wave	91 dead in Milwaukee County	Multi-agency heat plan; 11 heat deaths in 1999 (attribution weak)
1997–98	Strong El Niño: southern Wisconsin 6–8°F above normal; Lake Michigan maximum ice cover ~15%	No deaths found	n/a
2015–16	Strong El Niño: 4–5°F above normal, low snow	No deaths found	Housing First (2015) already under way
Oct 2019	Tent City under I-794: state eviction deadline slipped into snow	n/a	Encampment cleared later (lens, not re-verified)
30–31 Jan 2019	Polar vortex during a weak El Niño; −20°F	38-year-old woman dead in an apartment with a failed thermostat; a Cudahy man dead in his yard	No cold plan found
2020–23	Medical Examiner hypothermia deaths, countywide	12, 11, 10, 9	Flat even as unsheltered homelessness fell
Thanksgiving 2021	19°F, the season's first shelter night	72-year-old dead near Wisconsin Ave & Old World 3rd, one block west of the river	n/a
12–15 Jan 2024	Cold snap during the strong 2023–24 El Niño	3 unhoused men dead: under a bridge, in a car, by a heating vent	Shelters “at capacity and beyond”
27 Nov – 13 Dec 2024	Early season, just after the warming season opened on ~25 Nov	5 probable hypothermia deaths, ages 56–82; 3 homeless	Every-night season through March continued

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
Jan 2026	−40°F wind chill	None found	Every downtown site west of the river
2022 → 2025	Fatal overdoses, countywide	674 → 387	Opioid settlement funds deployed

The recurrence nobody has noticed: Milwaukee wrote a plan for heat after 1995 and has none on record for cold. And both recent cold-death clusters came inside El Niño winters (weak in 2019, strong in 2024), the winters the forecast calls mild. Hypothermia deaths held at about 10 a year while unsheltered homelessness fell sharply, which suggests some of the risk is indoors, among people with no heat.

WHERE THE EXPERTS DISAGREE

UNRESOLVED

- **Overdose is the threat** (Public health, Philanthropist) versus **cold is under-prepared** (Emergency manager, Skeptic, Historian, Frontline). Public Health warns against “cold-snap theatre” that diverts overdose resources. This report ranks the action that serves both first: naloxone in warm rooms.
- **A mild El Niño winter will probably lower cold mortality** (Public health, citing Gasparrini) versus **it lowers guard and cold snaps still kill** (Skeptic, Historian). Both can be true. Fewer cold days overall, but deaths cluster on the nights the system is not watching.
- **Build a Cold Task Force** (Historian) versus **add cold triggers to what exists** (verifier). Unresolved until someone confirms whether Milwaukee's existing plans already cover cold.

WHAT NOBODY COVERED

BLIND SPOT

The largest cold toll is probably invisible: **cardiovascular and respiratory deaths among older housed people**, which Gasparrini's method attributes to moderate cold but which no certificate records as weather. No lens could measure it for Milwaukee. No lens counted **power-dependent residents** in Juneau Town's ZIPs (53202) through HHS emPOWER. Only the Historian raised the **Lake Michigan shoreline** along Juneau Park, where low ice cover in El Niño winters leaves more open water for storm waves. That risk is to infrastructure rather than lives, and it was not verified. No lens looked at **visitors and workers** downtown, or at the skywalk network as winter shelter.

* Suggested formulation: check with a health worker before sharing.

HOW TO READ THIS METHOD

The panel is author-built. Seven simulated expert lenses were used to generate these recommendations. Where they agree, that is a strong hypothesis, not a consensus of the field. Every recommendation was then independently checked for evidence, transferability to this location, and feasibility before the peak; the verdict tags carry the result.

Mortality figures are resolved at Climate Impact Lab impact-region scale, and are not neighbourhood figures. Milwaukee County holds about 920,000 people, well above the typical ~300,000 region, so the containing region may be finer than the county; it was inferred, not resolved. The El Niño projection covers *heat only*; cold, overdose, and outages are carried here by the record rather than by a published number.

HOW THIS BRIEFING WAS PRODUCED METHODOLOGY

- **Scope.** The location was taken as given: Juneau Town, the historic east-of-the-river downtown of Milwaukee, Wisconsin, from the Milwaukee River to Lake Michigan between Juneau Avenue and I-794. Climate Impact Lab impact region, as this report records it: Milwaukee County, WI, *inferred* rather than resolved on the Impact Map. Where lenses used administrative data, the unit is named: Milwaukee County (Medical Examiner deaths, overdoses, point-in-time count) and a commercial "Downtown" profile with an undefined boundary (age). None is Juneau Town, which holds about 1% of the county. Several deaths cited sit one block west of the river, outside Juneau Town but inside the same downtown network.
- **Exposure.** Climate Impact Lab, *451,000 at Risk* (September 2026, pre-peer-review): ECMWF SEAS5 seasonal forecasts (51-member ensemble) applied to temperature–mortality relationships across 24,378 regions, against a 1996–2025 baseline, for June 2026 – February 2027. It covers heat only. It is never added to the Impact Map's long-run projections or quoted from the map's GDP-denominated mortality.
- **Expert panel.** Seven simulated expert lenses (emergency manager, public health scientist, frontline responder, skeptic, economist, historian, and philanthropist) each researched this place independently and in parallel, from web sources, to a fixed brief.
- **Reconciliation.** Duplicate actions were merged and backing lenses recorded. Convergence was tested for shared bias before it counted. Here the six-lens convergence on a temperature trigger was demoted: the lenses agreed because they read the same pre-2024 news coverage, and verification found the sites already open every night. Contradictions were kept, not resolved by dropping a side. Actions were ranked by expected lives saved per unit of effort, each was given a named payer or marked unfunded, and the funding placements were reconciled against the ranked list.

* Suggested formulation: check with a health worker before sharing.

- **Verification.** Five separate verifier agents checked 12 recommendations and the four funding placements for evidence, transferability, and feasibility. Financial actions were also checked for availability, payer, trigger, and existing implementation, and the envelope for real recipients, published unit costs, and duplication, then re-summed. The cleat action was not re-verified here; it carries the Lake Geneva run's same-day verification of the same trial.
- **Verdicts.** *Confirmed*: evidence, transferability, and feasibility all hold. *Partially confirmed*: holds after the corrections shown. *Unverified*: could not be checked to a primary source. *Contraindicated*: would do harm or waste the window here.
- **Adaptation Inventory.** Not re-opened in this run. The inventory's US filter (19 entries) was read in a browser earlier the same day for the Gaslamp Quarter briefing, and those titles were used here. *Substance abuse regulation policies* (Pavanello & Zappalà 2024) bears loosely on action 1 and *Change in forecast quality* (Shrader et al. 2023) indirectly on action 6. No US entry covers warming centres, outreach, or utility reconnection.
- **Health wording.** Passages where a wording error could hurt someone (naloxone and overdose) are marked "* Suggested formulation: check with a health worker before sharing." They were checked against published sources, not by a health professional. They are suggestions, not medical advice, and anyone passing them on should confirm them with a doctor, pharmacist, nurse, or community health worker first. The marker was added on 2 November 2026.
- **Rules.** No cost is derived by dividing a published total without the division being shown and labelled. The $800-per-place figure is $100k ÷ 125 places, shown. No local figure is invented; where data is thin, the report says so.
- **Limits.** The panel and verifiers are AI research agents working from web-available sources, not human experts, and no one in this place was interviewed. **The shared web-search budget was exhausted during verification**, so verifiers fetched pages directly, and city.milwaukee.gov, county.milwaukee.gov, and umos.org returned 403 errors. Medical Examiner data, City cold-weather policy, the County opioid-settlement figures ($101M net, $34M committed), and the Milwaukee WHEAP agency were therefore not confirmed against primary records. No 2025 or 2026 unsheltered count was found.

References · verified by Phase 5 unless marked. Warming sites every night through March 2026, FOX6 (verified) · 2024 warming plan, FOX6 (verified) · Overnight warming centres, Milwaukee Magazine (verified) · 2024 thresholds, NNS (verified) · Jan 2026 sites, Journal Sentinel via Yahoo (lens, not re-verified) · Jan 2024 deaths, WTMJ (lens, not re-verified) · ME hypothermia counts, TMJ4 (lens, not re-verified) · Late-2024 deaths, Wisconsin Examiner (lens, not re-verified) · Thanksgiving 2021 death, TMJ4 (lens, not re-verified) · Jan 2019 apartment death, AP (lens, not re-verified) · Jan 2025 fall and hypothermia, FOX6 (lens, not re-verified) · Wisconsin DHS Naloxone Direct (verified) · Wisconsin DHS naloxone access (verified) · Walley et al., BMJ 2013 (verified) · Coffin & Sullivan 2013 (corrected) · Goedel et al. 2019, cold and overdose (verified) · 2025 fatal overdose data, Milwaukee County (lens; site returned 403) · Gasparrini et al., Lancet 2015 (verified) · Zhang et al. 2019, Toronto

* Suggested formulation: check with a health worker before sharing.

hypothermia (verified) · Weisskopf et al., AJPH 2002 (corrected) · PSC 113 (verified) · WHEAP and HVAC programme (verified) · Open Door Café (verified) · Street Angels (corrected) · Repairers of the Breach (corrected) · McKiernan 2005, cleats (verified, Lake Geneva run) · NCHS Data Brief 532 (verified, Lake Geneva run) · El Niño snow, CBS58 (lens, not re-verified) · 2015–16 winter review, WisDOT (lens, not re-verified) · Tent City, WPR (lens, not re-verified) · 1995 heat wave, NWS (lens, not re-verified)

Exposure data · Climate Impact Lab, 451,000 at Risk · Impact Map · Adaptation Inventory (US)

Generated 8 October 2026 for Juneau Town, downtown Milwaukee. El Niño Ready is volunteer-run and is not affiliated with Climate Week NYC, The Climate Group, or Columbia University.

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